# Furosemide

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| **Scope of Practice** | P |
| **Classification** | • Loop Diuretic |
| **Usual Presentation** | • 20mg/2ml ampule |
| **Mechanism of Action** | • Inhibits sodium reabsorption in the ascending loop of Henle in the kidneys resulting in increased urinary output and fluid excretion. |
| **Indications** | • Acute pulmonary edema with signs of fluid overload |
| **Contraindications** | • Liver Cirrhosis<br>• Anuria |
| **Precautions** | • Pulmonary edema without symptoms of fluid overload. |
| **Side Effects** | • Hypotension<br>• Increased urine output<br>• Dehydration<br>• Electrolyte imbalances |
| **Adult Dose & Administration** | • 40mg IV (slow over 1-2 minutes) |
| **Pediatric Dose & Administration** | • Not for pediatric use |
| **Use in Pregnancy and Lactation** | • Pregnancy: May cause fetal harm. Administration in the prehospital setting is not advised.<br>• Lactation: Minimal infant risk, If essential to be administered, the mother should be advised to stop breast feeding until advised by their physician. |
| **Additional Information** | • Furosemide should be administered only if signs of fluid overload are present.<br>• Administration of non-fluid overloaded patients typically is highly associated with in-hospital dehydration and electrolyte imbalances and has not demonstrated significant improvement in patient outcomes. |